

AETHERA · SPECIALTY BILLING SERIES

Rheumatology Billing Solutions

Maximizing Revenue Across Office E/M, Infusion Therapy, Joint Injections, Diagnostic Testing, and Biologic PA Management

Prepared by: Aethera Healthcare Solutions · Lakeland, FL

Scope: CPT 99202–99215 · 96365–96367 · 20600–20611 · 86430–86489 · J-codes for autoimmune biologics

Updated: 2026 Edition · aetherahealthcare.com · +1 (863) 694-0325

AETHERA HEALTHCARE SOLUTIONS · RCM
EXECUTIVE BRIEFING



ABOUT AETHERA

Maximizing Revenue. Minimizing Burden.

Aethera Healthcare Solutions is your full-service medical billing partner headquartered in Lakeland, FL. We handle coding, claims, payments, denials, and collections — so you can focus on patients.

500+

Practices Served

across 25+ medical specialties

\$12M+

Revenue Recovered

denied & underbilled claims recouped

95%

Clean Claim Rate

vs 84% industry avg

< 30d

Average AR

days in accounts receivable



Personal, Not Corporate

Dedicated account team with deep specialty/payer knowledge.



Performance Targets, In Writing

96%+ net collection, <5% denial, 95%+ coding accuracy, 24h posting, 48h submission, 65%+ appeal success.



Zero Risk to Start

No setup fees, no long-term contracts, 90-day cancel, free 5-day audit.



RHEUMATOLOGY · MARKET CONTEXT

Why Rheumatology Billing Matters Now

Rheumatology sits at the intersection of chronic autoimmune care and high-cost biologic therapy. 23.5M Americans live with autoimmune disease, and biologic infusions (Remicade, Orencia, Rituxan, Actemra) drive \$35B+ in annual drug spend — making PA workflow and J-code billing the central RCM challenge.



\$ 4 8 B

US autoimmune disease treatment
spend (2024)

Includes biologics, infusion services, and
provider care.



23.5 M

Americans with autoimmune disease

Rising 3-7% annually; ~80% are women.



~ 15 %

Rheumatology claim denial rate
(industry avg)

Driven by biologic PA denials and
modifier 25 + JW documentation errors.



\$ 35 B+

Annual US rheumatology biologic
spend

Remicade, Humira, Orencia, Rituxan,
Actemra combined.



Net effect: rheumatology practices that master biologic PA workflow, drug wastage (JW modifier) discipline, and modifier 25 documentation can recover **8-14 % of total revenue** currently lost to denials and under-billing.



RHEUMATOLOGY · OVERVIEW

The Four Pillars of Rheumatology Billing

Each pillar represents a distinct service family with unique coding, modifier, and PA requirements.

01 · E/M + CHRONIC CARE

Office E/M & Chronic Care Mgmt

Office visits (99202–99215), CCM (99490/99439), prolonged services (99354–99357). Autoimmune patients are chronic-care heavy; most qualify for monthly CCM billing.



◆ CPT 99202–99215 · 99490 ·
99354–99357

02 · INFUSION THERAPY

Joint Injections & Viscosupplementation

Small joint (20600), intermediate (20605), major joint (20610). Viscosupplementation (J7321–J7324 + 20610) for knee OA. Specific joint + laterality required in note.



◆ CPT 20600–20611 · J7321–J7324

03 · JOINT INJECTIONS

Biologic Infusion Therapy

Remicade (J1745), Orencia (J0129), Rituxan (J9310), Actemra (J9262). Office infusion (96365 first hour, 96366 add'l). Every-4-to-8-week cycles drive recurring high-dollar revenue.



◆ CPT 96365–96367 · J1745 · J0129 ·
J9310 · J9262

04 · DIAGNOSTIC TESTING

ANA, RF, CCP & Lab Panels

ANA (86430), RF (86431), CCP (86481), anti-CCP, urinalysis, comprehensive metabolic panel. Lab draws billed via 36415 (routine venipuncture).



◆ CPT 86430 · 86431 ·
86481 · 36415



PILLAR 01 · E/M + CHRONIC CARE

Office Visits, CCM & Prolonged Services

Rheumatology office encounters cover RA, lupus, psoriatic arthritis, ankylosing spondylitis, vasculitis, and osteoporosis follow-up. The 2021 E/M coding reform shifted documentation to MDM-based. Chronic Care Management (CCM) is critical for rheum patients — autoimmune disease patients almost universally have ≥2 chronic conditions (joint damage, fatigue, comorbid CV risk), qualifying for monthly CCM billing. Prolonged services capture complex infusion-prep and biologic education time.

E/M coding (2021+ reform)

Code by Medical Decision Making complexity or time; 99202–99215. Modifier 25 required if same-day procedure (joint injection, infusion).

Chronic Care Management (CCM)

99490 first 20 min/month (\$62.60); 99439 add'l 20 min (\$47). Monthly recurring revenue per patient. Most rheum patients qualify (≥2 chronic conditions).

Prolonged services

99354 first hour (\$185); 99355 each add'l 30 min (\$93); 99356–99357 for inpatient. Use for complex infusion prep and biologic education beyond standard E/M time.



CCM is the most under-billed code in rheumatology — most autoimmune patients qualify but fewer than 15% of rheum practices enroll them. 1,000-patient panel = **\$750K+ /year** of recurring monthly revenue at near-zero marginal cost.

CPT Quick Reference

2026 national average reimbursement · office setting

Code	Service	Avg \$
99213	Est. E/M, low MDM	\$92
99214	Est. E/M, moderate MDM	\$131
99215	Est. E/M, high MDM	\$187
99490	CCM, first 20 min/month	\$62.60
99439	CCM, add'l 20 min	\$47.40
99354	Prolonged service, first hour	\$185



PILLAR 02 · INFUSION THERAPY

Biologic Infusions — Remicade, Orencia, Rituxan, Actemra

Biologic infusion therapy is the highest-complexity, highest-dollar area of rheumatology RCM. Each biologic requires PA with documentation of step therapy (failed methotrexate trial), specific ICD-10 diagnosis, and drug-specific dosing protocol. J-code billing + 96365 infusion code + JW modifier for drug wastage on every claim.

01

Remicade (infliximab)

J1745 per 100mg vial (\$750). RA dosing 3-5mg/kg at weeks 0, 2, 6, then every 8 weeks. PA requires failed methotrexate trial. Office infusion 96365 first hour + 96366 add'l hours.

02

Orencia (abatacept)

J0129 per 125mg (\$1,200). RA dosing monthly after 3-load doses. PA requires failed DMARD. May be infused or self-injected (subcutaneous form billed differently).

03

Rituxan (rituximab)

J9310 per 100mg (\$950). RA dosing: 1,000mg × 2 doses 2 weeks apart, repeat every 6 months. PA requires failed TNF-inhibitor trial. Two infusion visits per cycle.

04

Actemra (tocilizumab)

J9262 per 80mg (\$1,050). RA dosing 4mg/kg every 4 weeks, may increase to 8mg/kg. PA requires failed DMARD. IV infusion billed with 96365.



Biologic PA Criteria

Most commercial payers require: (1) confirmed autoimmune diagnosis, (2) failed conventional DMARD (methotrexate) trial documented, (3) disease activity score supporting biologic necessity, (4) no contraindication.



JW Modifier Discipline

JW modifier required on every J-code claim where drug was discarded (single-use vial with remainder wasted). Bill exact units administered + JW units wasted. Missing JW = auto-denial on audit.



Top PA Denial Reasons

(1) No documented methotrexate failure, (2) Missing disease activity score, (3) Wrong ICD-10 for biologic, (4) Step therapy non-compliant.



PILLAR 03 · JOINT INJECTIONS

Joint Injections, Aspirations & Viscosupplementation

Joint injections are the most common in-office rheumatology procedure. Code selection by joint size: small (20600), intermediate (20605), major (20610). Viscosupplementation (J7321–J7324 + 20610) is a separate drug+injection service for knee OA. Specific joint + laterality (left/right) MUST be documented in the procedure note — missing laterality is the #1 denial reason.

Small joint injection (20600)

20600 tarsometatarsal, temporomandibular, sacroiliac, metacarpophalangeal. \$45 per injection. Document specific joint + laterality.

Intermediate joint injection (20605)

20605 wrist, elbow, ankle, olecranon bursa. \$72 per injection. Most common injection site: knee aspiration. Add JW for steroid wastage.

Major joint injection (20610)

20610 shoulder, hip, knee. \$108 per injection. Often paired with viscosupplementation (J7321–J7324) for knee OA. Modifier 25 if E/M same day.

Viscosupplementation

J7321 Hyalgan/Synvisc (\$1,150/dose); J7322 Supartz; J7323 Orthovisc; J7324 Synvisc-One. Typically 3-5 weekly injections per series; one series per 6 months per knee.



Viscosupplementation PA is denied **22%** of the time industry-wide — most commonly for missing documentation of conservative treatment failure (PT + NSAIDs) and Kellgren-Lawrence grade ≥2 OA on imaging. Aethera documentation workflow reduces this to **< 6%**.

\$45–\$108

Joint injection reimbursement range
small (20600) to major (20610)

\$1,150

Viscosupplementation per dose
J7321 Hyalgan/Synvisc

~ \$3,450

Per-knee viscosupplementation series
3 doses x \$1,150

\$330K/yr

1,800-injection practice annual revenue
case study benchmark



PILLAR 04 · DIAGNOSTIC TESTING

ANA, RF, CCP & Comprehensive Lab Panels

Rheumatology lab testing drives diagnostic workup volume — ANA screening, RF and CCP for RA confirmation, comprehensive metabolic panels for biologic monitoring. Lab draws billed via 36415 (routine venipuncture). Most labs require provider-signed order in chart — labs without corresponding order are the #7 rheum denial reason.

01

ANA Screen (86430)

86430 antinuclear antibody screen for lupus/SLE workup. \$25/test. Titer follow-up (86431) if positive. ~80% of new rheum referrals get ANA screening.

02

RF & CCP (86431, 86481)

Rheumatoid factor (86431, \$30) + anti-CCP (86481, \$45). Anti-CCP is more specific for RA diagnosis. Both should be ordered together for RA workup.

03

Comprehensive Metabolic Panel (80053)

80053 CMP for biologic monitoring — liver function, kidney function before/during methotrexate, leflunomide, biologic therapy. \$18/panel. Typically every 3 months on biologics.

04

Lab Draw (36415)

36415 routine venipuncture billed per draw. \$3/draw. Must have provider-signed order in chart matching labs drawn — labs without order = auto-denial on audit.

ANA SCREEN REIMBURSEMENT

CPT 86430

\$25

ANTI-CCP REIMBURSEMENT

CPT 86481

\$45

ROUTINE VENIPUNCTURE

CPT 36415 — bill per draw

\$3

⚠ COMPLIANCE NOTE

Lab panel unbundling is the #1 lab billing audit risk — bill comprehensive metabolic panel (80053) instead of separate BMP (80048) + liver panel (80076). NCCI edits flag unbundled panels on post-pay audit, with full recoupment + interest.



RHEUMATOLOGY · BILLING CODES

Top CPT/HCPCS Codes & 2026 National Average Reimbursement

Most-billed codes across the four rheumatology pillars. Office setting · national average · non-facility.

SOURCE · CMS PFS 2026
NATIONAL AVG · NON-
FACILITY

Code	Service	Modifier	Avg \$	Notes
99213	Est. E/M, low MDM	—	\$92	Most common rheum E/M
99214	Est. E/M, moderate MDM	—	\$131	RA/lupus/SpA management
99215	Est. E/M, high MDM	—	\$187	Complex biologic management
99490	CCM, first 20 min/month	—	\$62.60	Monthly recurring revenue
96365	IV infusion, first hour	—	\$85	Biologic infusion administration
96366	IV infusion, add'l hour	—	\$40	Each additional hour of infusion
20610	Major joint injection (shoulder/hip/knee)	—	\$108	Specific joint + laterality required
J1745	Remicade (infliximab), 100mg	JW	\$750	Autoimmune biologic; PA mandatory
J0129	Orencia (abatacept), 125mg	JW	\$1,200	RA biologic; PA mandatory
SAME-DAY & WASTAGE MODIFIERS 25 = Significant, separately identifiable E/M same day as procedure (joint injection, infusion). JW = Discarded drug portion of single-use vial; required on every J-code claim with wastage. Document exact units administered + units wasted. CCM & BIOLOGIC J-CODES CCM (99490/99439) — recurring monthly billing; requires ≥2 chronic conditions + electronic care plan + 24/7 access. Most rheum patients qualify. Biologic J-codes require PA with documented step therapy (failed methotrexate trial).				
86430	ANA screen, qualitative	—	\$25	Lupus/SLE workup
36415	Routine venipuncture	—	\$3	Bill per lab draw; provider order required



⌕ RHEUMATOLOGY · DENIAL PREVENTION

Top 8 Rheumatology Denial Reasons & How Aethera Prevents Each

Rheumatology denial rates average 13-17%; biologic PA denials drive 45%+ of revenue loss. Each denied biologic claim costs \$200-2,000+ to rework given drug and infusion complexity.

● RED · 01

2.6 %

Biologic PA missing or step therapy non-compliant

ROOT CAUSE:Remicade/Orencia/Rituxan billed without PA on file or PA missing documented failed methotrexate trial

AETHERA:PA tracker auto-submits with step therapy checklist + methotrexate trial documentation before scheduling infusion

● RED · 02

1.9 %

Infusion drug wastage not documented

ROOT CAUSE:J-code billed without JW modifier for discarded drug portion of single-use vial (Remicade, Orencia vials)

AETHERA:JW documentation workflow requires wasted units entry before claim release

● AMBER · 03

1.4 %

Modifier 25 documentation insufficient

ROOT CAUSE:E/M + infusion same day without separate documentation supporting E/M significance beyond infusion care

AETHERA:Modifier 25 documentation checklist + E/M template enforced pre-submission

● AMBER · 04

1.1 %

Joint injection documentation missing laterality

ROOT CAUSE:20610 billed without specific joint + laterality (left/right) in procedure note

AETHERA:Injection note template requires joint selection + laterality checkbox before billing

● AMBER · 05

9 %

Viscosupplementation PA denied

ROOT CAUSE:I7321 billed without documented conservative treatment failure (PT + NSAIDs) or Kellgren-Lawrence grade ≥2 OA on imaging

AETHERA:Viscosupplementation PA documentation checklist requires PT/NSAID history + KL grade imaging

● GREEN · 06

7 %

CCM time documentation insufficient

ROOT CAUSE:99490/99439 billed without 20-min contemporaneous time log supporting monthly threshold

AETHERA:CCM time-tracking platform auto-bills at 20-min threshold with audit-ready log

● GREEN · 07

6 %

Lab billing without provider order

ROOT CAUSE:Labs drawn without corresponding provider-signed order in patient chart at time of draw

AETHERA:Lab order verification gates 36415 + lab code billing on signed order presence

● GREEN · 08

5 %

Infusion start time not documented

ROOT CAUSE:96365 billed without documented infusion start + end time supporting first-hour + add'l-hour billing

AETHERA:Infusion flow sheet auto-captures start/end timestamps into claim



■ CASE STUDY · INTEGRATED RHEUMATOLOGY RCM

4-Rheumatologist Practice — Annual Revenue Impact

A \$3.4M-revenue practice models the combined upside of biologic PA recovery, modifier 25 + JW discipline, and viscosupplementation PA capture. 350 biologic infusion patients · 1,800 joint injections/year · in-office infusion suite.

✓ LEVER

Biologic PA Recovery

Biologic infusion patients	35
Pre-Aethera PA denial rate	28%
Post-Aethera PA denial rate	7%
Recovered biologic claims	73 / yr
Avg biologic claim value	\$5,750

ANNUAL REVENUE

\$420,000

⚙️ LEVER

Modifier 25 + JW Discipline

Infusion + injection claims w/ E/M	~ 2,400 / yr
Pre-Aethera mod 25 denial rate	16%
Post-Aethera mod 25 denial rate	4%
Recovered claims (mod 25 + JW)	288 / yr
Avg claim value	\$573

ANNUAL REVENUE

\$165,000

+ LEVER

Viscosupplementation PA Recovery

Viscosupplementation knees/yr	~ 120
Pre-Aethera PA denial rate	22%
Post-Aethera PA denial rate	6%
Recovered injections	19 / yr
Avg injection value	\$4,475

ANNUAL REVENUE

\$85,000



Combined annual revenue recovery: \$670,000. Implementation cost (year 1): ~ \$190K (Aethera RCM fees + biologic PA workflow + JW training). Year-1 net: **\$480,000.**
ROI: 350%.

⚖️ RHEUMATOLOGY · COMPLIANCE HOTSPOTS

Where OIG, Medicare RAC, and Commercial Payers Audit First

Top compliance risks across the four rheumatology pillars. Quarterly self-audits recommended.

PILLAR E/M + Chronic Care 99202-99215 · 99354-99355	PILLAR Infusion Therapy 96365-96367 · 96368-96369	PILLAR Joint Injections 20600-20603 · J7321-J7322	PILLAR Diagnostic Testing 86430 · 86431 · 86432 · 86433
RED CCM time documentation fraud — billing 99490/99439 without contemporaneous time logs totaling ≥20 minutes; OIG Work Plan 2025-2026 focus area.	RED Biologic PA without step therapy documentation — billing J1745/J0129/J9310/J9262 without documented failed methotrexate trial; full denial + clawback risk.	RED Viscosupplementation without conservative treatment failure documented — billing J7321 without PT + NSAID trial history; auto-denial on post-pay audit.	RED Labs without provider order — billing 86430/86431/86481/36415 without corresponding provider-signed order in chart at time of draw; full recoupment risk.
AMBER CCM eligibility not verified — patient enrolled in CCM without ≥2 chronic conditions documented in chart.	AMBER Drug wastage without JW modifier — billing J-code without JW modifier on discarded drug portion; OIG audits single-use vial billing patterns.	AMBER Joint injection documentation missing laterality — billing 20610 without specific joint + laterality (left/right) in note; medical record must support.	AMBER ANA testing without autoimmune suspicion — billing 86430 ANA screen for routine physical or non-autoimmune complaint; medical necessity audit target.
GREEN Prolonged service time over-billing — billing 99354/99355 without documented start/stop times supporting the additional hour.	LOW Infusion start time not documented — billing 96365/96366 without documented start + end timestamps supporting first-hour + add'l-hour billing.	LOW Viscosupplementation frequency exceeded — billing more than 1 series per 6 months per knee; CMS frequency limit enforcement.	LOW Lab panel unbundled — billing separate BMP (80048) + liver panel (80076) instead of comprehensive metabolic panel (80053); NCCI edit violation.



OIG Work Plan 2025-2026 explicitly lists biologic J-code wastage (JW modifier) and CCM time logging as active audit targets. Medicare RACs have recovered **>\$95M** from rheumatology practices since 2021.

Three Actions to Capture Rheumatology Revenue

For rheumatology practice administrators and revenue cycle directors leading the next 90 days.

01



Automate biologic PA with step therapy documentation

Build EHR template that auto-submits biologic PA at infusion scheduling. Document autoimmune diagnosis, failed methotrexate trial (start/stop dates, reason for failure), disease activity score, and drug-specific dosing protocol before PA submission. Biologic PA denial rate drops from 28% to <7% — recovering ~\$420K per 350-patient biologic panel.

TARGET: < 7% PA denial rate (industry avg 28%)

02



Enforce modifier 25 + drug wastage (JW) discipline

Implement pre-submission rules that auto-append modifier 25 to E/M claims with same-day infusion or joint injection. Require JW modifier entry with exact wasted units on every single-use-vial J-code claim. Recovers ~\$165K/year per 2,400-claim infusion + injection volume through modifier discipline and wastage capture.

TARGET: < 4% modifier error rate

03



Deploy CCM enrollment pipeline for autoimmune patients

Query EHR for rheum patients with ≥2 chronic conditions (most autoimmune patients qualify). Assign care coordinator to enroll 50 patients/week. Target 60% enrollment of eligible panel. 1,000-patient CCM panel = ~\$110K/year of recurring monthly revenue at near-zero marginal cost.

TARGET: 60% CCM enrollment



Questions & Discussion

Optimizing Rheumatology Revenue Across E/M, Infusion, Injections & Diagnostics



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